[redacted]

प्रतिक्रिया फॉर्म/FEEDBACK FORM

[redacted]

[illegible]

Did you avail the medical treatment under AB PM-JAY scheme?

☑ हाँ / Yes

☐ नहीं / No

2. क्या आप अस्पताल में मिले इलाज से संतुष्ट हैं?

Are you satisfied with the treatment you received at the hospital?

☑ हाँ / Yes

☐ नहीं / No

3. क्या आयुष्मान भारत मित्र हॉस्पिटलाइजेशन के दौरान आपको जानकारी और सहायता प्रदान की थी?

Did Ayushman Mitra Arogya Mitra (PMAM) provide you with necessary information and assistance during your hospitalization?

☑ हाँ / Yes

☐ नहीं / No

[illegible]

☑ हाँ / Yes

☐ नहीं / No

4. क्या आपने स्वास्थ्य सेवा का लाभ लेने के लिए अस्पताल को अपनी जेब से कोई भुगतान किया?

Did you pay any money from your pocket to the hospital for the healthcare service you availed?

☐ हाँ / Yes

☑ नहीं / No

5. क्या अस्पताल ने आपके इलाज के लिए पर्याप्त दवाएं उपलब्ध कराई थीं?

Did hospital provide sufficient medicine for the treatment you availed?

☑ हाँ / Yes

☐ नहीं / No

7. क्या अस्पताल ने आपके ठहरने के दौरान अस्पताल में अच्छी किस्म की गुणवत्ता-युक्त भोजन की व्यवस्था की थी?

Did the hospital serve you with quality food during the stay at the hospital?

☑ हाँ / Yes

☐ नहीं / No

अन्य अतिरिक्त प्रतिक्रिया / Any other feedback:

[redacted]

लाभार्थी का हस्ताक्षर / Beneficiary Signature

मोबाइल नंबर/Mobile Number

[redacted]

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DISCHARGE SUMMARY

[redacted]

[redacted]		Age/Gender	: 1Y/Female
Guardian	[redacted]	Admit Date & Time	: 11/03/2020 2:30 PM
UHID	: [illegible]	Discharge Date	: 18/03/2020 11:00 AM
Address	: [redacted]	Contact No	: [redacted]
Bed Type	:	Patient Category	: AYUSHMAN BHARAT(PMJAY CMJAY)
Company Name	:	Bed Number	: PICU BED -1

Chief Complaint :
- Cough and intermittent fever since last 2 weeks
- Did not pass stool for consecutive 4 days following which enema was given (outside)

History of Illness :
- Patient presented with above mentioned complaints
- H/O vomiting induced by persistent cough
- WIDAL TEST- Salmonella typhi O ve H - negative

On Examination :
- Conscious, rational
- PR- 125 bpm
- RR- 37 b/m
- SpO2- 89 % in RA
- Temp- 97.8 degree F
- Chest- NAD
- CVS- NAD
- P/A-NAD

Investigation : Enclosed

Diagnosis : Enteric fever

Treatment Type : Medication

Treatment Details :
1. INJ ROCE 500MG IV/BD X 4 DAYS
2. INJ AMIKACIN 150MG IV/OD X 4 DAYS
3. SYP ZYTP 5ML OD X 4 DAYS
4. SYP TUMMYSOFT 5ML BD X 4 DAYS

Condition on Discharge : HR- 84/MIN, RR- 18/M,SPO2- 96%,TEMP- F,CNS-NFT,CVS-NA,P/A- NAD

Advice on Discharge

1.SYP TAXIM FORTE (20MG/KG)
5ML TWICE DAILY 7 DAYS
2.SYP ZYTP
5ML ONCE DAILY 10 DAYS
3.SYP TUMMY SOFT
5ML TWICE DAILY AFTER FOOD 5 DAYS
4.SYP CHLZ M
5 ML AT BED TIME 5 DAYS
5.LOCAL LOTION APPLY 2 TIMES 5-7 DAYS

Follow-up : Follow up after 7 days or SOS

Special Note (If any) :

[redacted]

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DISCHARGE SUMMARY

[redacted]

Guardian

[redacted]

Age/Gender

Admit Date & Time

Discharge Date

[redacted]

Patient Category

Bed Number

[redacted]

[redacted]

[redacted]

Signature

[redacted]

[redacted]

[redacted]

[redacted]

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[redacted]

[redacted] JAKOTI, TRIPURA – 799279

Patient Na: [redacted] Age/Sex: 17 YR 0M 0D / F
DOA : 11/03/2026 00:00 PM
Consultan: [redacted]

UHID NO.:

Ward:

IP NO.:

Age / Sex:

Bed No.:

MEDICATION CARD

Drug Name:								
INJ. XONE		6		11/3/26	12/3/26	13/3/26		
	Route	10						
Dose	IV	12			Pampi	Pampi		
500MG	Time	14						
	BD	18	Sist	Anindita				
Date & Time	Dr. Sign	22		5:30pm	Kimi	Kimi		
11/3/26		24		5:30pm				

Drug Name:								
INJ. AMIKACIN		6						
	Route	10						
Dose	IV	12			Pampi	Pampi		
150MG	Time	14						
	OD	18	Sist	Anindita				
Date & Time	Dr. Sign	22		5:30pm				
11/3/26		24						

Drug Name:								
SYP. ZN-20		6						
	Route	10			Pampi	Pampi		
Dose	P/O	12						
5ML	Time	14						
	OD	18	Sist	Anindita				
Date & Time	Dr. Sign	22		5:30pm				
11/3/26		24						

Drug Name:								
SYP. TUMMYSOFT		6						
	Route	10			Pampi	Pampi		
Dose	P/O	12						
5ML	Time	14						
	BD	18	Sist	Anindita				
Date & Time	Dr. Sign	22		5:30pm	Kimi	Kimi		
11/3/26		24						

Drug Name:								
SYP Montaelc		6						
	Route	10						
Dose	P/O	12						
5ml	Time	14						
	HS/OD	18						
Date & Time	Dr. Sign	22				Kimi		
13/3/26		24						

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Drug Name:				13/3/26	14/3/26				[redacted]
NEBU NS		6			Vani				
	Route	10							
Dose	Nebulisation	12							
3ml	Time	14							
	BD	18	Staff Chandan						
Date & Time:	Dr. Sign	22							
13/3/26		24							

Date & Time:									
		6							
	Route:	10							
Dose		12							
	Time:	14							
		18							
Date & Time:	Dr. Sign	22							
		24							

Drug Name:									
		6							
	Route	10							
Dose		12							
	Time	14							
		18							
Date & Time:	Dr. Sign	22							
		24							

Drug Name:									
		6							
	Route	10							
Dose		12							
	Time	14							
		18							
Date & Time:	Dr. Sign	22							
		24							

Drug Name:									
		6							
	Route	10							
Dose		12							
	Time	14							
		18							
Date & Time:	Dr. Sign	22							
		24							

Drug Name:									
		6							
	Route	10							
Dose		12							
	Time	14							
		18							
Date & Time:	Dr. Sign	22							
		24							

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[redacted]

[redacted]

UHID : UH-15678
Age/Sex: 1Y 9M 0D / F
Name DOA : 11/03/2026 02:30 PM Sex : M / F
IP No : IP/23/1393
UHID : [redacted] Bed No.
Consultant :

INVESTIGATION REPORTS CHART

Date	11/3/26							
HEMATOLOGY								
Hb%	11.1							
Pcv%								
Tc / Cmm								
ESR								
RBC	4.15							
Platelets	5.63							
BT mins								
CT mins								
INR								
PT sec								
PTT sec								
Neutrophils	50							
Lymphocytes	41							
Monocytes	05							
Basophils	00							
Eosinophils	04							
MP Smear								
BIOCHEMISTRY								
RBS mg/dl								
FBS mg/dl								
PPBS mg/dl								
Urea	14.2							
Creatinine	0.26							
Na	139.0							
K	4.0							
Chloride								
Phosphate								
Calcium								
Bicarbonate								
Magnesium								
Uric Acid								
Total Protein								
Albumin								
Globulin								
Bilirubin Total								
Bilirubin Indirect								
SGOT								
SGPT								
GGT								
Alkaline Phos								
LDH								
Cholesterol								
LDL								
HDL								
VLDL								
Triglycerides								
SEROLOGY	HIV			HBsAg			HCV	
Informed By :								
Received By :								
Informed To :								

INVESTIGATION REPORTS CHART

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MICROBIOLOGY

SPECIMEN	COLLECTION DATE	GRAM STAIN	PRELIM REPORT	FINAL REPORT
URINE				
BLOOD				
SPUTUM				
OTHERS				

ECHO	USG
Signature Date :	Signature Date :
CT / MRI	OTHERS
Signature Date :	Signature Date :

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[redacted]

[redacted]
UHID : UH-15678
Age/Sex: 1Y 9M 0D / F
DOA : 11/03/2026 02:30 PM
IP No : IP/25/1383
[redacted]

IED CONSENT FOR CRITICAL CARE / HIGH RISK PATIENT

_____ Age: _____ Admn. No: _____ UHID: _____
_____ Date of Admission : _____ Ward / Bed : _____

INSTRUCTION; This Consent form should be signed by the patient (if he / shehan adult 18 years or older,conscious& oriented having ability to make an informed decision); otherwise by a biological percent / legal guardian if the patient in minor, or by the spouse or adult children or percent or adult brother or sister or other family member or custodian or friend or Employer or Significant other (in this orderof priority).

1. It has been explained to me in language which I understand that my Patient _____ UHID. No. _____ requires to be admitted in CRITICAL CARE UNIT for monitoring and critical care of his / her medical condition.
2. I understand that the condition of my patient is not stable at this time, it may deteriorat rapidly during treatment and require following procedures:

- CENTRAL VENOUS ACCESS
- VENTILATION
- (LIFE SAVING) DO NOT REQUIRE
- TEMPORARY PACE MAKER INSERTION
- ARTERIAL LINE
- ICD
- OTHERS

3. All these procedures though life saving can have complications.
4. Knowing all this, I give my consent to the Doctors of UNAKOTI HOSPITAL'S Critical Care Unit to treat my patient.
Understanding that the staff of this hospital are working for the betterment of my patient, I shall in no w hold them responsible for the ultimate outcome.

NAME :

SIGNATURE : [redacted] DATE &TIME 11/03/26
THUMB IMPRESSION
WITNESS : _____ RELATIONSHIP: [redacted]

DOCTOR'S SIGNATURE : [redacted] DTAE & TIME 11.03.26

[redacted]

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[redacted]

[redacted]

UHID : UH-15678
Age/Sex: 1Y 9M 0D / F
DOA : 11/03/2026 02:30 PM
IP No : IP/25/1302
[redacted]

PATIENT & FAMILY EDUCATION

..... Age/Sex : ..... Admn. No : ..... UHID No : .....

..... Date of Admission : ..... Ward/Bed .....

DATE & TIME	EDUCATION /INFORMATION PROVIDED	TO WHOM	PATIENT/P'T. ATTENDENT SIGN	COUNSELOR'S NAME, SIGN & DESIGNATION
11.3.26	Pt party counselled about possible infection & need for admission	[redacted]		
12.3.26	IV antibiotics given for infection present, pt party counselled			
13/3/26	Condition of the pt has been explained to the pt. party			

...ple chart may also be used for Special / Tailor-made Consent. To be filled by the person who educates ... Consultant, RP, RS, RMO/MO, Administrator, Counselor, Dietician, Physiotherapist, etc

[redacted]

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[redacted]

ADMISSION / CONSENT FOR

Patient's Name (In Block Letters): [redacted]

Age: 1 y 9 m Sex: F

Name of Son/Daughter/Wife/Husband: [redacted]

Religion: Hindu Occupation: Nationality: Indian

Address: [redacted]

P.O: [redacted] P.S: [redacted]

State: [redacted]

Phone No (Res): Office:

Mobile (1) : (2)

Next of Kin / Guardian Name: [redacted] nship with the Patient [redacted]

Address of Guardian: [redacted]

Occupation: Designation (If Applicable):

Office Address:

Phone No (Res): Office: Mob [redacted]

Registration No: 2144 - 15678 Date of Admission: 11/2/25

Time of Admission: 2:30 PM Admitting Doctor's name [redacted]

Mode of Payment: Cash/ Card/ RTGS/NEFT/Google pay/UPI

In case of any emergency person to be contacted

Name: [redacted] Relation: [redacted]

Mob no: Alternate No:

Address: [redacted]

I [redacted] do hereby give full consent knowingly for admission in ward / cabin / icu / nicu or any type diagnostic, therapeutic of any others procedures to be performed on me/ my / patient by the Doctors/ Sisters or any medical professional of [redacted] I have been explained with full clarity in a known language, which is clear to me on the admission or procedure to be performed on me / my patient. I am fully conscious about the result / risk factors of such disease or treatment orprocedures decided by the Doctors of this Centre. I shall not hold the staff and the Doctors responsible for any type of unfortunate consequences during the treatment process for myself/ my patient. I/ we authorize [redacted] for all medicine supply from this centre or out source. The complications, if any due to medicine / consumable / blood product sourced from outside the center, are the responsibility of mine.

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[redacted]

[redacted]

I agree and also give my consent to the following as mentioned below:

a) Deposit shall be made at regular intervals as and when required in addition to the initial deposit. All deposit will be adjusted in the final bill at the time of discharge.

b) [redacted] ALshall not be responsible for any loss of valuables or patient.

c) In absence of Consultant Physician / Visiting specialist, the Resident Doctor and qualified nurse will look after my patient as per protocol.

d) The Patient and guardian shall jointly and severally be liable for payment of final bill before discharge and accept legal obligation thereof.

e) Monitoring by competent physician or general physician/medicine specialist or medical officer in both physical and online mode will be acceptable to the patient and relatives.

f) In case condition of my patient become severe or in case of any emergency situation or leave against medical advice (LAMA),I / We will be liable for my patient to be shifted to higher center or hospital as decided/referred by Doctors (or decided by us in case of LAMA).The center will not be liable for bed arrangement of further treatment.

g) Bed charges, Medicine, investigations, Consultation charges of Physician and specialist, Nursing & RMO charges, consumable, oxygen will be charged additionally.

h) Included bed charge with complimentary snacksand packaged drinking water.

i) It is implied that the doctor may order necessary investigations or procedures up to Rs. 5000(five thousand) without any additional consent. This may include Ryle's tube insertion, foley's catheterization, IV channel, etc.

j) 80% of the bill has to be cleared daily by 4pm.

k) The details of admission procedure have been explained to me in a language that I/We understand / local language.

[redacted]

Next Kin / Guardian

Person responsible for Payment

[redacted]

Attending Officer

[redacted]

Relationship with Patient

11/3/26 at 2:30 Pm

Admission time & Date

I, the undersigned have explained the details, nature/risks of the procedure performed and its possible and alternatives to the patient/legible representative. I am confident that he/she has understood the information fully as described in this documents.

Explained by

particular	Signature	Full name	Date	Time
Doctor	[redacted]	[redacted]	11/3/26	

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[redacted]
UHID : UH-15678
Age/Sex: 1Y 9M 0D / F
DOA : 11/03/2026 02:30 PM
IP No : IP/25/1382
[redacted]

INITIAL ASSESSMENT BY DOCTOR

CASE HISTORY & PHYSICAL EXAMINATION

Patient Name: [redacted] IP: .....

UHID no.: 15678 Dr. Name: [redacted] DOA: 11/03/26 Time of Arrival: .....

Vital Signs:

Temperature: 97.2°F Pulse: 123 /min. BP: — mmHG

Respiration: 27 /min CBG: 92 mg/dl Height: 76 cm Weight: 9.3 kg

HISTORY TAKEN FORM

Chief Complaints	History of intermittent fever since last 2 weeks. Diffuse pain abdomen for 2 days following which pain pain subside the hospital.
History of Present Illness	Patient presented with the above mentioned complaints. H/O: Vomiting induced by persistent cough
Drug History	NA
Past Medical & Surgical History	NA
Family & Marital & Obstetric History	NA
Physical Examination & Clinical Findings	Conscious & alert Chest: B/L clear. CVS: S1 S2 (+) PA: NAD SpO2: 92%
Investigation Ordered	Enclosed.

Contd...P/2

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[redacted]

INITIAL ASSESSMENT BY DOCTOR
CASE HISTORY & PHYSICAL EXAMINATION

Preventive Aspect of Care	
Diet & Regimen	NPM till further advice.
Provisional Diagnosis	Bifrontal Hem
Plan of Care	Medication.
Desired Outcome	
DISCHARGE PLANNING (Identify possible needs at home after discharge)	Social Support: Yes / No Home Equipment: Yes / No Physiotherapy: Yes / No Wound Care: Yes / No Any other need anticipated
Expected Date Of Discharge	
Examination By Dr. (FULL SIGNATURE)	[redacted] Date & Time of Assessment: [illegible]
Counter Signed By Treating Doctor (FULL SIGNATURE)	[redacted] Date & Time:

[redacted]

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[redacted]

[redacted]

MBBS, FICM, NBE(Delhi)DCH

Paediatrics & Neonatologist Consultant and Critical Care,

[redacted]

Patients Name:	[redacted]	AGE-1Y 9M	Sex-F	UHID-15678
Address:-	EGT	Mobile No:	7028051323	DATE-11/03/2026
Father Name	JESTMINA DARLONG	Mother Name	[redacted]	

Enteric Fever

wt. 8.8 kg

Bmd - 2.4 kg

o/e - Lethargic / sick look

intakite

cvs - clean B/L

CNS / RPAD | NAD

Typhoid on PICU

① Inj Xone 500 mg — IV BD

② Inj Amikacin - 150 mg IV OD

③ IVF DNS — 500 ml IV over 12 hourly in intermittently

④ Syp 2n-20 — 5 ml — OD

DR

(5) Syp Tummy soft
5ml — BD (after food)

(6) Temp monitoring 6 hourly

Dr [illegible]

Advice
CBC / CRP
Blood culture
and urine R/E
Stool R/E

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[redacted]

PICU

11/3/26
4:30PM.

No perio events.

O/E: Baby is comfortable but irritable.
Afebrile.

PR: 123 bpm.
RR: 37/min
SpO2: 93% in RA.
Temp: 97°F.

Chest: clear
CVS: S1S2 (+)
P/A: BS (+), soft.

Rx: CSF.

[redacted]

12.3.26
(2:10pm)

S/B Rm. Rounds (RMO)

C/E - Baby is irritable & crying, Lower lid stye (R) eye x 5 days
Last passed stool 2 days back [first normal, then loose]

11.3.26
T0 - 14.4K

PR - 120 bpm
RR - 29/min
SpO2 - 96% RA
Temp - 98.1°F
TS - DBF + SBI ++
TO - 3 times urine passed

Chest - clear
P/A - BS +, soft.
CVS - S1S2 +
Rx - warm compression (R) eye
[Inj. Cefo 500mg IV BD - Day 2
Inj. Amikacin 150 mg IV OD - Day 2]

[redacted]

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PICU

[redacted]

UHID : [redacted]
Age/Sex: 1Y 9M 1D / F
[redacted] PM

13/3/26
5:20 AM.

c/o: cold & cough.

[redacted]

tonsillitis, irritability ↓.
Afebrile.

PR: 122 bpm
RR: 30/min
SpO2: 93%
Temp: 97°F.

Chest: Rhonchi ++
CVS: } WNL.
PA: }

T/1: 250ml + DBF
T/2: 7% urination.
1x stool passed.

[redacted]

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[redacted]

CHILDREN WARD

13.3.20
(8:00 pm)

S/O- Dr. Gupta (RMO)

c/o No fresh complaints except cough & cold

O/E- Conscious, Irritable, afebrile

PR - 130/min
RR - 29/min
SpO2 - 96% RA
Temp 97.8°F

Chest- Min wheeze (+)
CVS / RS
PA
P/A

13.3.20

✓ Nebn with NS 3ml. → 1 hourly

✓ Sy CR12-M — 3ml — OD at

[redacted]

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[redacted]

DOCTOR'S PROGRESS NOTE

MRD No.:	Patient Name:	IP NO.:
Age: Sex:	Bed / Ward No.:	Dr. Name

[redacted]
UHID : UH-15678
Age/Sex: 1Y 9M 1D / F
DOA : 11/03/2026 02:30 PM
IP No : IP/25/1862
[redacted]

CHILDREN WARD

14/08/2026

(DC) Enteric fever

(1) Syp Taxim — Forte (50mg/kg)
5ml —— BD x 7Days

(2) Syp Zn-SO — 5ml — OD
x 10Days

(3) Syp Tummy soft
5ml —— 2 times after food
x 5Days

(4) Syp CPM — 5ml at night
x 5Days

(5) Linical Cotton
2 times x 5-7 days — Dr. Sul

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[redacted]

Case down

Patient conscious & oriented

Afebrile

Pulse – 128/min

RR – 28/min

Temp – 98°F

CNS – RBL AIE (+)

CVS – S1S2 (+)

Adv

CSF

Rm

[redacted]

VITAL SIGNS CHART

Date	Time	Blood Pressure	Temp	Resp	Pulse	SPO2	Consciousness Level	Sign
[illegible]	5AM	—	[illegible]	[illegible]	[illegible]	[illegible]	—	
	9AM	—	[illegible]	[illegible]	[illegible]	[illegible]	—	[illegible]
	10AM	—	[illegible]	[illegible]	[illegible]	[illegible]	—	[illegible]
	11AM	—	[illegible]	[illegible]	[illegible]	[illegible]	—	
	12AM	—	[illegible]	[illegible]	[illegible]	[illegible]	—	
	1AM	—	[illegible]	[illegible]	[illegible]	[illegible]	—	
	2AM	—						

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[redacted]

Date	Time	Pressure	Temp	Resp	Pulse	SPO2	Conscious Level	Sign
13/3/26								
	5AM	—	98.7°F	20b/m	6'b/m	96%	13/15	Pampi
	9AM	—	98.2°F	22b/m	126b/m	97%	"	
	10AM	—	98.5°F	24b/m	126b/m	98%	"	
	11AM	—	98.2°F	21b/m	130b/m	97%	"	
	12pm	—	98.1°F	22b/m	134b/m	98%		
	1pm	—	97.2°F	21b/m	130b/m	98%	"	
	2pm	—	98.2°F	20b/m	113b/m	98%	"	
	3pm	—	98.2°F	16b/m	140b/m	95%	"	
	4pm	—	97.6°F	22b/m	144b/m	96%	"	
	5pm	—	98.6°F	24b/m	140b/m	97%	"	Chandana UH-87
	6pm	—	97.8°F	22b/m	130b/m	97%	"	
	7pm	—	98.8°F	24b/m	132b/m	97%	"	
	8pm	—	98.8°F	24b/m	130b/m	97%	"	
	9pm	—	98.3°F	26b/m	130b/m	98%	"	
	10pm	—	97.8°F	26b/m	128b/m	97%	"	
	11pm	—	97.1°F	28b/m	135b/m	96%	"	
14/3	12am	—	98.1°F	24b/m	130b/m	96%	"	Kimi UH-86
	1am	—	98.6°F	23b/m	118b/m	97%	"	
	2am	—	97.6°F	28b/m	121b/m	95%	"	
	3am	—	98.8°F	24b/m	117b/m	97%	"	
	4am	—	98.3°F	23b/m	118b/m	98%	"	
	5am	—	98.4°F	26b/m	119b/m	98%	"	
	6am	—	96.8°F	28b/m	120b/m	96%	"	
	7am	—	98.3°F	18b/m	122b/m	97%	"	

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[redacted]

Patient Name: Baby of [redacted]
UHID : [redacted]
Age/Sex:
DOA : 11/[redacted]
In [redacted]

Consultant: [redacted]

Ward: [redacted]

[redacted] NO.:

Age / Sex:

Bed No.: [redacted]

[redacted] SIGNS CHART

Date	Time	Blood Pressure	Temp	Resp	Pulse	SPO2	Conscious Level	Sign
12/13	5 Am	—	98.6°F	32 b/m	130 b/m	96%	15/15	
	9 Am	—	98.2°F	32 b/m	131 b/m	96%	15	
	10 Am	—	98.2°F	30 b/m	130 b/m	96%	15	
	11 Am	—	98.3°F	30 b/m	136 b/m	97%	15	
	12 Pm	—	98.2°F	31 b/m	129 b/m	96%	15	Ramp 15°
	1 pm	—	98.2°F	29 b/m	120 b/m	96%	15	
	2 pm	—	97.6°F	30 b/m	126 b/m	97%	15	
	3 pm	—	97.6°F	31 b/m	121 b/m	97%	15	
	4 pm	—	98.4°F	29 b/m	123 b/m	97%	15	
	5 pm	—	98.4°F	31 b/m	120 b/m	97%	15	[redacted]
	6 pm	—	98.4°F	29 b/m	129 b/m	97%	15	97%
	7 pm	—	98.4°F	30 b/m	128 b/m	97%	15	
	8 pm	—	98.4°F	30 b/m	121 b/m	97%	15	
	9 Pm	—	98.2°F	32 b/m	125 b/m	98%	15	
	10 Pm	—	97.6°F	30 b/m	130 b/m	97%	15	
	11 Pm	—	97.8°F	32 b/m	132 b/m	98%	15	
12/13	12 Am	—	98.4°F	32 b/m	130 b/m	96%	15	
	1 Am	—	98.8°F	30 b/m	131 b/m	97%	15	
	2 Am	—	98.4°F	29 b/m	128 b/m	97%	15	
	3 Am	—	98.4°F	30 b/m	122 b/m	98%	15	
	4 Am	—	98.3°F	34 b/m	128 b/m	98%	15	[illegible]
	5 Am	—	98.7°F	32 b/m	130 b/m	98%	15	44/-86
	6 Am	—	98°F	32 b/m	120 b/m	97%	15	
	7 Am	—	98.2°F	30 b/m	122 b/m	98%	15	

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[redacted]

Patient No: Age/Sex: 1Y 9M (0) / F
DOA : 11/03/2025 02:30 PM
Consultant: [redacted]

UHID NO.: Ward:
IP NO.: Age / Sex: Bed No.:

VITAL SIGNS CHART

Date	Time	Blood Pressure	Temp	Resp	Pulse	SPO2	Conscious Level	Sign
11/03/25								
	3pm	—	97.8F	37/min	123/min	92%	Conscious	Amiodipin
	4pm	—	97.6F	36/min	122/min	94%	~	
	5pm	—	97.6F	36/min	123/min	97%	~	
	6pm	—	97.8F	34/min	123/min	96%	~	Amiodipin 17.8
	7pm	—	98.6F	36/min	121/min	97%	~	
	8pm	—	97.8F	36/min	122/min	97%	~	
	9pm	—	97.8F	38/min	120/min	97%	"	
	10pm	—	98.4F	34/min	128/min	98%	"	
	11pm	—	98.6F	38/min	120/min	97%	"	
11/3	12am	—	98.2F	34/min	125/min	98%	"	
	1am	—	97.4F	30/min	126/min	95%	"	
	2am	—	98.3F	36/min	128/min	96%	"	
	3am	—	98.6F	30/min	120/min	97%	"	Mami 14.8
	4am	—	98.8F	32/min	130/min	95%	"	
	5am	—	97.6F	34/min	135/min	95%	"	
	6am	—	98.5F	34/min	132/min	96%	"	
	7am	—	97.5F	32/min	125/min	95%	"	

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CHILDREN WARD

[redacted]

Age/Sex: 1Y 9M 1D / F
DOA : 11/03/2026 02:30 PM
NAME_ IP No : [redacted]
UHID_ [redacted]

14/03/26

OPE

INTAKE / OUTPUT CHART

_____ AGE_____ SEX_____ WARD_____

	INTAKE				OUTPUT				
	ORAL/HYDRATION		PARENTERAL						
DATE/TIME	TYPE	AMOUNT TAKEN	TYPE	AMOUNT INFUSED	URINE	VOMITING	DRAINAGE	STOOL	REMARKS / SIGN
14/03									
5 AM	F40	20 ml							
9 AM	BMT	100 ml			✓				
10 AM									
11 AM	DPF								
12 AM					✓				
1 AM	DPF								
2 AM									

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CHILDREN WARD

[redacted]

[redacted]

Age/Sex: 1Y 9M 20 / F
DOA : 11/03/2026 02:30 PM
IP No : [redacted]

13/3/26

INTAKE / OUTPUT CHART

NAME

UHID

AGE SEX WARD PICU

DATE/TIME	INTAKE: Oral/Intravenous Type	Amount Taken	Ryles/Drains Type	Amount Refused	OUTPUT: Urine	Vomitch	Drainage	Stool	Insensible loss
12/3/26									
5AM	Breastfed	20ml							
9AM	ORS	25ml			✓				
10AM	DBF	[illegible]							
11AM	ORS	10ml							
12PM	DBF								
1PM	ORS	30ml			✓				
2PM	Lunch	40ml							
3PM	DBF								
4PM									
5PM	ORS	20ml			✓				
6PM									
7PM	DBF								
8PM	ORS	15ml			✓				
9PM	Dinner	30ml							
10PM	DBF								
11PM	H2O	10ml							
12AM	DBF								
1AM	ORS	10ml			✓				
2AM									
3AM	DBF								
4AM									
5AM	DBF								
6AM	H2O	10ml			✓				
7AM	DBF								

Total Intake - 235ml + DBF

Total output - 6 times urine passed.

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[redacted]

DOA : 11/03/2026 02:30 PM
IP No : IP/25/1382
[redacted]

AGE ____ SEX ____ WARD ____
CONSULTANT ____

ITAKE

DATE/TIME	ORAL/PARENTERAL TYPE	ORAL/PARENTERAL AMOUNT TAKEN	PARENTERAL TYPE	PARENTERAL AMOUNT INFUSED	OUTPUT URINE	ASPIRATION	DRAINAGE	STOOL	VOMITING/SKIN
12/3/26									
8 AM	[illegible]	30 ml							
9 AM	ORS	30 ml			✓				
10 AM	DBF						✓		
11 AM	DBF								
12 PM	ORS	30 ml							
1 PM	ORS	20 ml			✓		✓		
2 PM	DBF								
3 PM	lunch	30 ml							
4 PM	ORS				✓				
5 PM	"							passed	
6 PM	DBF				✓				
7 PM	ORS	20 ml							
8 PM									
9 PM	ORS	20 ml							
10 PM	Dinner	35 ml							
11 PM	DBF				✓				
12 AM	ORS	10 ml							
1 AM									
2 AM									
3 AM	DBF				✓				
4 AM									
5 AM	DBF								
6 AM					✓				
7 AM	H2O	15 ml							

Total Intake - 220 ml + DBF
urine passed - 7 times urine passed.
stool passed - 1 times stool passed.

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[redacted]

Patient Name: [illegible] 11:55 00 / F
DOA: 11/03/2025 02:30 PM
IP No : IP/25/1383
Consultant: [redacted]

UHID NO.:
IP NO.:
Ward:
Age / Sex:
Bed No.:

I. & OUTPUT CHART

NASOGASTRIC & ORAL INTAKE					PARANTERAL INTAKE			OUTPUT			
Date & Time	Type of fluid	Oral	N/G	IV	Type of IV fluid	Amount in ML	IV Med in ML	Urine	NG Aspiration	Vomiting	Others
11 0736					1 vial DNS						
3pm	DBF					41ml					
8pm						41ml					
1am						41ml		✓			
5am	DBF					41ml					
3am						41ml					
5pm	DBF					41ml					
9pm	DBF					41ml					
10pm	Lemonade	300ml				41ml					
11pm						41ml					
12am	DBF					41ml		✓			
1am	ORS	100ml				41ml					
2am						41ml					
3am	DBF					41ml					
4am						Stop					
5am	DBF										
6am	H2O	30ml						✓			
7am											

Total Intake - DBF + 583ml
Urine passed - 3 time passed
Stool passed - Nil

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CHILDREN WARD

[redacted]

NAME DOB : 11/03/2020 02:30 PM
IP No :
UHID
ADMISSIO
DIAGNOSIS

AGE SEX WARD DATE
INWI
CHIEF COMPLAINTS

		Morning	Evening	Night	Totals
INTAKE					
OUTPUT					
HYGIENE	PERSONAL UNIT & SURROUNDINGS	Bathing			TOILERS MORNING
INVESTIGATIONS	DONE	NIT			→ D/C today.
	DUE	NO			
	PENDING REPORTS	NO			
MEDICINE	NEW	NO			
	STOP	NO			
	CHECK IO BANDS	YES			
	IV FLUIDS	NO			
					EVENING
REFERRAL		NO			
PROCEDURE OR SURGERY	DONE	/ NIL			
	DUE				
	PLAN				
DIET	TYPE	DIET +			
	SPECIAL INSTRUCTION	SOFT DIET			
					NIGHT
ALERT / INCIDENTS		NO			
IMPORTANT MESSAGES		I.V Fluids PR			
STAFF NAME		[illegible]			
EMPLOYEE ID		1411			
HANDOVER TIME		8am - 2pm			

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CHILDREN WARD

[redacted]

[redacted]

[redacted]

Age/Sex: 1Y 9M 1D / F
DOA : 11/03/2020 02:30 PM

NAME _____

UHID _____ [redacted]

ADMISSION DATE / POD _____

DIAGNOSIS _____

		MORNING	EVENING	NIGHT	TIME
INTAKE					
OUTPUT					
HYGIENE	PERSONAL UNIT & SURROUNDINGS	maintain	maintain	maintain	
INVESTIGATIONS	DONE	NO	NO	NO	
	DUE	NO	NO	NO	
	PENDING REPORTS	NO	NO	NO	
MEDICINE	NEW	NO	NO	NO	
	STOP	NO	NO	NO	
	CHECK BY NAKSH	yes	yes	yes	
	N'FLUENTS	NO	NO	NO	
REFERRAL		–	–		
PROCEDURE OR SURGERY	DONE	nil	nil	nil	
	DUE				
	PLAN				
DIET	TYPE	DBF+SIB	DBF+SIB/DBF		
	SPECIAL INSTRUCTION		Ondemand		
ALERT / INCIDENTS		–	–		
IMPORTANT MESSAGES		RR orientation	RR maintain	check vital sign	
STAFF NAME		Pooja	Chandana	Mini	
EMPLOYEE ID		180	87	86	
HANDOVER TIME		6AM to 2pm	8 pm	8PM	

SCHEDULE
MORNING
EVENING
→ stool not passed 12/03/20 last stool passed 12/03/20 at 5 pm.
NIGHT
plan D/C tommorow.

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[redacted]

NAME DOA : 11/03/2026 02:30 PM AGE SEX WARD PIC DATE 12/03/26

UHID [redacted]

ADMISSION DIAGNOSIS CHIEF COMPLAINTS

DIAGNOSIS

		MORNING	EVENING	NIGHT
INTAKE		200 ml		80ml + DBF
OUTPUT		3 stool		5 time urine passed.
HYGIENE	PERSONAL, UNIT & SURROUNDINGS	Maintain	maintain	maintain
INVESTIGATIONS	DONE	NO	YES	NO
	DUE	YES	NO	NO
	PENDING REPORTS	NO	YES	YES
MEDICINE	NEW	NO	NO	NO
	STOP	NO	NO	NO
	CHECKED BANDS	YES	YES	YES
	IV FLUIDS	NO	NO	NO
REFERRAL			NO	
PROCEDURE OR SURGERY	DONE	NO	NO	Nil
	DUE	NO		
	PLAN	NO		
DIET	TYPE	DBF + S/O	DBF +	DBF / SOFT
	SPECIAL INSTRUCTION		SOOT diet + ORS	diet / ORS
ALERT / INCIDENTS		3/o chest crackles	NO	3/o chest maintain
IMPORTANT MESSAGES		RR - chest maintain	I.P upper RR	
STAFF NAME		Pooja	Anjali	Rimi
EMPLOYEE ID		150	178	144-56
HANDOVER TIME		8 AM to 2pm	2pm - 8pm	8 AM

REMARKS

MORNING

-> stool R/E due

EVENING

-> stool R/E Report due.

NIGHT

stool R/E report collected

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[redacted]

[redacted]

Passion For Caring

HANDOVER

[redacted]

UHID : UH-15078
Age/Sex: 1Y 9M 0D / F
DOA : 11/03/2026 02:30 PM
IP No : IP/25/1382

[redacted]

AGE_____ SEX_____ WARD_____ DATE_____

CONSULTANT_____

CHIEF COMPLAINT_____

		MORNING	AFTERNOON	NIGHT	TIME (HR)
INTAKE					
OUTPUT					
HYGIENE	PERSONAL, UNIT & SURROUNDINGS		Maintain.	maintain	REMARKS / MORNING
INVESTIGATIONS	DONE		yes	NO	
	DUE		NO	yes	
	PENDING REPORTS		yes	yes	
MEDICINE	NEW		yes	yes	
	STOP		NO	NO	
	CHECKED MARDI		yes.	yes	
	IV FLUIDS		On flow	yes	
					EVENING
REFERRAL			NO		→ Urine R/E, stool R/E
PROCEDURE OR SURGERY	DONE				
	DUE				
	PLAN		NIL	nil	
DIET	TYPE		DBF	DBF, ORS, S/D	
	SPECIAL INSTRUCTION		enderman.		
					NIGHT
ALERT / INCIDENTS			check vitals	check I/O chart	stool R/E due
IMPORTANT MESSAGES			check Temp.	check Temp	
STAFF NAME			Chandana	vlmi	
EMPLOYEE ID			87	UH-86	
HANDOVER TIME			8 pm	8Am	

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[redacted]

NURSING CARE PLAN

OD — 14/03/26

	GOODS TO ACHIVE	NURSING INTERVENTIONS	EVALUATION / OUTCOME
• NO break complain	• improve patient health status.	• Assess the patient general condition. • I/O and vitals are monitoring and recorded. • All the medication given as per Dr advised. • DBF + SOL — given	• patient health status are improved. plan D/C today.
[redacted]	DATE 14/03/26	[redacted]	TIME 8AM — 2AM.

CHILDREN WARD

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[redacted]

NURSING CARE PLAN

(N) 13/3/26

	GOODS TO ACHIVE	NURSING INTERVENTIONS	EVALUATION / OUTCOME
Pt has cold & cough related disease condition	→ To improved pt condition	→ Assess the baby health condition. → Vital sign checked and recorded. → All medication given as per dr adviced. → I/O chart monitoring → DBF / soft diet → Stool not passed → Urine passed.	→ vital sign is stable.
ALLOCA [redacted]	DATE 13/3/26	[redacted]	TIME 8am

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(E) [redacted]

NURSING CARE PLAN

PROBLEM IDENTIFIED	GOODS TO ACHIVE	NURSING INTERVENTIONS	EVALUATION / OUTCOME
c/o patient having cold & cough related to disease condition.	To improving the patient condition.	→ Assess the patient general condition. → vital are check & recorded. → I/O chart maintain → All medication given as per DR. advice. → S/D continue ondemand & OBF	→ vital are ~~steady~~ stable.
[redacted]	DATE 13/3/26	SIG[redacted]	TIME 8.pm

CHILDREN WARD

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[redacted]

NURSING CARE PLAN

12/3/20.
Morning duty 7-2

	GOALS TO ACHIEVE	NURSING INTERVENTIONS	EVALUATION / OUTCOME
- do patient cold & cough.	- To Improved the patient Cinomatal condition.	- Patient hand over taken from night duty staff. - patient all due medication give. as per dr. Order - patient Vitals check & Record. - patient Feeding & S/O ~~tat~~. - patient urine & Stool passed.	- patient is Stable now.
AL [redacted]			TIME 8 AM to 2 pm

CHILDREN WARD

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[redacted]

	GOALS TO ACHIEVE	NURSING INTERVENTIONS	EVALUATION / OUTCOME
pt has general weakness of dehydration related to disease condition	→ To improved pt condition.	→ Assess the baby health condition. → vital sign checked & recorded. → All medication given as per dr adviced. → I/O chart monitoring → Soft diet, ORS & DBF.	→ Baby pt is on treatment.
ALLOCATION STAFF [redacted]	DATE 17/12/26	SI [redacted]	TIME 5 pm

PICU

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[redacted]

NURSING CARE PLAN

[redacted]G
UHID : UH-15678
Age/Sex: 1Y 9M 1D / F
DOA : 11/03/2026 02:30 PM
IP No : IP/25/1382
[redacted]

MRD No.:		IP NO.:	Dept./Unit:
Age:	Bed / Ward No.:	Dr. Name:	

PROBLEM	GOAL	INTERVENTIONS	EVALUATION / OUTCOME
• poor nutritional status related to disease condition as evidence by patient health status as ovs. observation	• To improve patient health status.	• Assess the patient general condition. • I/O and vitals are monitoring and recorded. • All the medication given as per Dr advised • DBF + soft diet + ORS given.	• patient health status are improving to before.
ALLOCATED NAME: [illegible]	DATE: 12/03/26	SIGNATURE: [illegible]	TIME: 2 pm — 8 pm

PICU

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[redacted]

NURSING CARE PLAN

12/8/26
Morning duty staff

MRD No.: | Patient Name: | IP NO.: | Dept./Unit:

Age: | Sex: | Bed / Ward No.: | Dr. Name:

PROBLEM IDENTIFIED	GOAL/TO ACHIEVE	NURSING INTERVENTIONS	EVALUATION / OUTCOME
- the patient fever Chilg c	To Improved the patient Condition.	- patient hand over taken from night duty staff - patient all due morning medication give as per dr order - patient vitals check Q Based - patient stat & urin passed	- patient is stable mode

ALLOCATING NURSE NAME | DATE 12/8/26 | [redacted] | TIME 8 AM to 8 pm

PICU

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[redacted]

MRD No.:	Patient Name:	IP NO.:	Dept./Unit:
Age: Sex:	Bed / Ward No.:	Dr. Name:	

PROBLEM IDENTIFIED	GOAL TO BE ACHIEVE	NURSING INTERVENTIONS	EVALUATION / OUTCOME
pt has fever & irritable related to disease condition	→ to improved pt. condition	→ Assess the baby health condition. → vital sign checked & recorded. → All medication given as per dr advised. → I/O chart maintain. → check temp → DBF, ORS & soft diet.	→ pt is on treatment
ALLOCATION BY SUBNAME: Vemi	DATE: 11/5/26	SIGNATURE: Vemi	TIME: 8am

PICU

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[redacted]

Patient Name:	IP NO.:	Dept./Unit:
[redacted]	Bed / Ward No.:	Dr. Name:

[redacted]	GOALS TO ACHIEVE	NURSING INTERVENTIONS	EVALUATION / OUTCOME
C/o patient having fever related to disease condition as evidance by checking vitals.	To improving the patient condition.	→ Assess the patient condition. → vital are check & Recorded. → All medication given as per Dr. Advice. → DBF ondemen. → I/O chart maintain	→ vital are Stable
ALLOCATION STAFF NAME: Chandana	DATE: 11/8/26	SIGNATU [redacted]	TIME: 8.00

picu

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[redacted]

Name	[redacted]	Age/Gender	: 1Y 9M 0D/ Female
UHID	: UHF10078	Case No	: LAB25/0008
Patient Type	: OPD	Case Date	: 11/03/2026 04:19 PM
Ref. By	[redacted]	Sample Date	: 11/03/2026 04:19 PM
Contact No	: [illegible]	Report Date	: 11/03/2026 05:30 PM

[redacted]

Test	Result	Unit	Biological Ref. Range
CRP			
[Sample: Serum] [Method: Immunoturbidimetric]	1.76	mg/L	(0 - 6)

CRP plays a role in host defence and tissue repair.
There is a lag time of 8 - 10 hrs between stimulus and rise in serum levels, as compared to a lag time of 24-72 hrs, for other acute phase reactants.

Note: TEST DONE BY TURBIDIMETRIC

—: End of Report :—

Verified by

[redacted]

[redacted]

Dr Rahul Pal
MBBS, MD- Pathology

[redacted]

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[redacted]

Name	[redacted]	Age/Gender	: 1Y 9M 0D / Female
UHID	: OPP 13878	Case No	: LAB25/8009
Patient Type	: OPD	Case Date	: 11/03/2026 04:19 PM
Ref. By	[redacted]	Sample Date	: 11/03/2026 04:19 PM
Contact No	: [redacted]	Report Date	: 11/03/2026 05:30 PM

[redacted]

Test	Result	Unit	Biological Ref. Range
KIDNEY FUNCTION TEST (KFT)			
(Sample: Serum)			
BLOOD UREA	14.2	mg/dl	(15 - 45)
SERUM CREATINNE (Method: Kinetic)	0.23	mg/dl	(0.6 - 1.3)
SERUM URIC ACID (Method: UREASE)	3.5	mg/dL	(3 - 6.5)
SODIUM	139.0	mmol/L	(135 - 151)
POTASSIUM	4.0	mmol/L	(3.6 - 5.1)

—| End of Report |—

Verified by

[redacted]

Sr. PRASENJIT DAS
MBBS MD
[illegible]
Visiting Consultant Pathologist

Dr Rahul Pal
MBBS, MD- PATHOLOGY

[redacted]

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[redacted]

Name	[redacted]	Age/Gender	: 17 9M 0D/ Female
UHID	: UH-15578	Case No	: LAB25/6009
Patient Type	: OPD	Case Date	: 11/03/2026 04:19 PM
Ref By	[redacted]	Sample Date	: 11/03/2026 04:19 PM
Contact No	[redacted]	Report Date	: 11/03/2026 05:30 PM

[redacted]

Test	Result	Unit	Biological Ref. Range
CBC (COMPLETE BLOOD COUNT)			
(Sample: Blood)			
COMPLETE HAEMOGRAM			
HAEMOGLOBIN(Hb)	11.1	gm/dl	(12.5 - 16)
TOTAL LEUCOCYTE COUNT(TLC)	14400	/cumm	(4000 - 11000)
DIFFERENTIAL LEUCOCYTE COUNT(DLC))			
NEUTROPHIL	50	%	(40 - 75)
LYMPHOCYTES (Method: BC 3000 Plus cell counter)	41	%	(20 - 45)
EOSINOPHILS (Method: BC 3000 Plus cell counter)	04	%	(1 - 6)
MONOCYTE	05	%	(2 - 10)
BASOPHIL	00	%	(0-0)
RBC COUNT	4.15	Million/Cmm	(3.8 - 4.8)
PCV	32.4	%	35-48%
M.C.V	78.2	fL	(83 - 101)
M.C.H	26.7	pg	(27 - 32)
M.C.H.C	34.1	pg/dl	(32 - 37)
PLATELET COUNT	5.53	lakh/cmm	(1.5 - 4)
RDW-SD	46.9	fL	(35 - 56)
RDW-CV	16.7	%	(11 - 15)
MPV	9.1	fl	(8.5 - 12)
PDW	16.6		(9 - 17)

Verified By

[redacted]

[redacted]

Dr. [illegible]
MBBS, MD
[illegible] (TGHC)
Visiting Consultant Pathologist

Dr Sohan Pal
MBBS, MD- MICROBIOLOGIST

24/7
EMERGENCY SERVICE

No. 1 Hospital Chain in Tripura

[redacted]

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[redacted]

Name	[redacted]	Age/Gender	: 34 YR (M/ Female)
Patient Type	: OPD	Case No.	: [illegible]
Ref. By	[redacted]	Case Date	: 17/08/2026 04:15 PM
Contact No.	[redacted]	Sample Date	: 17/08/2026 05:28 PM
		Report Date	: 17/08/2026 02:29 PM

[redacted]

STOOL ROUTINE EXAMINATION

PHYSICAL EXAMINATION

COLOUR	:BROWNISH
CONSISTENCY	:SOLID
MUCUS	:SLIGHTLY PRESENT
VISIBLE BLOOD	:NOT SEEN

CHEMICAL EXAMINATION

REACTION(pH	:6.8
REDUCING SUGAR	:ABSENT

MICROSCOPIC EXAMINATION

PUS CELLS	:3-4/HPF
RBC	:1-2/HPF
MACROPHAGE	:ABSENT
OVA	:ABSENT
CYST	:ABSENT
[illegible] CELLS	:PRESENT
STARCH	:PRESENT
BACTERIAL FLORA	:PRESENT
FAT GLOBULES	:PRESENT

DR. PRASANJIT DAS
[illegible]
[illegible] (TGMC)
Visiting Consultant Pathologist

[redacted]

[redacted]

[redacted]

[redacted]

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[redacted]

[redacted]		Age/Gender	: 19 Yrs 10/ Female
: UH-13578		Case No	: LAB25/0069
: OPD		Case Date	: 11/08/2025 04:19 PM
: [redacted]		Sample Date	: 12/08/2025 02:42 AM
[redacted]		Report Date	: 12/08/2025 02:04 AM

[redacted]

BACTERIA : A FEW

OTHERS : ABSENT

<< End of Report >>

[redacted]

Scanned with OKEN Scanner

[redacted]

[redacted]		Age/Gender	: 19 YRS 2D/ Female
: U4-15678		Case No	: LAB25/6009
: OPD		Case Date	: 11/03/2026 06:19 PM
[redacted]		Sample Date	: 12/03/2026 01:02 AM
[redacted]		Report Date	: 12/03/2026 02:04 PM

[redacted]

URINE EXAMINATION TEST

PHYSICAL EXAMINATION

COLOUR	: PALE YELLOW
APPEARANCE	: SLIGHTLY HAZY
pH	: 7.0
DEPOSIT	: PRESENT

CHEMICAL EXAMINATION

SPECIFIC GRAVITY	: 1.015
PROTEIN (ALBUMIN)	: NIL
GLUCOSE	: NIL
BILE SALT	: ABSENT
BILE PIGMENT	: ABSENT
KETONE	: ABSENT
BLOOD	: ABSENT
NITRITE	: ABSENT

MICROSCOPIC EXAMINATION

PUS CELLS	: 3-4/HPF
RBCS	: NIL
EPITHELIAL CELLS	: 3-4/HPF
CRYSTALS	: ABSENT

[redacted]

Scanned with OKEN Scanner